



TR3203 Teaching Case

**NiaHealth: Scaling preventive
health without losing the human touch**

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In the late summer of 2025, Sameer Dhar sat across from his co-founders – Mike Goss, Tanya ter Keurs, and Saif Uddin Mahmud – staring at a familiar dashboard that, this time, felt unfamiliar. The numbers were not alarming. In fact, they were objectively strong. Revenue continued to grow steadily at eight to ten percent month-over-month. Member satisfaction scores remained above 90 percent. Operations were stable. The company, by most definitions, was succeeding.

And yet, something felt off. Growth was not compounding.

It had been around a year and a half since they started NiaHealth, a health tech company built to reimagine healthcare as a continuous, proactive system rather than a reactive one. The goal was simple: to help individuals understand, manage, and improve their health before problems arose. In a rapidly evolving tech ecosystem, building a healthtech company came with its own set of constraints, regulatory; clinical; and operational, but Sameer was driven by purpose. That sense of mission had carried him and the team through the early uncertainty, through product iteration, and through the initial traction that validated their idea.

Despite the successful monthly cohort onboardings and healthy revenue streams, the company's expansion did not exhibit the exponential trajectory Sameer and his team had projected. While growth existed, it lacked acceleration. What should have been a compounding engine instead resembled a series of isolated pushes forward.

For months, the co-founders had rationalized the slowdown. The summer period had been slower than usual. They anticipated a rebound in September, then January. But the data for the start of 2026 contradicted these assumptions. Growth no longer followed predictable seasonal patterns. Instead, it occurred in episodic bursts driven by individual deals rather than a repeatable engine. As Sameer later reflected, "We were advancing, but not in a way that would let us scale confidently. Every decision felt amplified by uncertainty."

By the end of the review session, Sameer and his co-founders were forced to confront an uncomfortable truth. They had achieved what many startups strive for, but none of it was enough for the stage they were entering. To become the company they envisioned, NiaHealth needed something far more elusive: repeatable, predictable, compounding growth. Sameer needed to achieve 15% month-over-month growth to become the company he wanted to be. Without it, even a well-run company risked quietly plateauing.

Sameer leaned back, weighing the paths ahead. They could double down on D2C marketing: pouring more resources into acquisition, scaling campaigns, and pushing harder on the funnel, hoping volume would unlock growth. Or they could turn inward, investing deeply in the product: refining the experience, strengthening the behavioral

loop, and making Nia so compelling that members stayed, engaged, and brought others in organically. Each path demanded something different – capital, time, conviction – and none came with certainty. The question wasn't just what to do next, but which bet was worth making before time ran out.

Origin story

The idea that would become NiaHealth did not begin in a lab, a hospital, or even a startup office. It began as a lingering discomfort – one that Sameer Dhar could not quite shake.

Sameer was born and raised in Alberta, Canada. In 2010, he enrolled at the University of Alberta, pursuing a Bachelor of Commerce in Finance. He went on to take on roles at Morgan Stanley, Alberta Investment Management Corporation, and Clairvest Group, where he learned to evaluate risk, allocate capital, and think in terms of scale. But over time, he began to feel a growing disconnect. The systems he was analyzing were efficient, but abstract. The impact of his decisions, while significant, was often several layers removed from the individuals they ultimately affected.

Still, Sameer's transition into healthcare was not immediate. It came gradually, through a series of questions that became harder to ignore. He found himself thinking, "Why did one of the most resource-intensive sectors in the world consistently produce suboptimal outcomes?" "Why were preventable conditions so prevalent?" "And why did interventions so often come too late?"

These questions led Sameer to found Sensasure, a health technology company focused on improving care in nursing homes for elderly with incontinence. He spent extended periods inside care facilities, actually living in the nursing homes, observing patients, interacting with caregivers, and witnessing firsthand how the system functioned on the ground. Over time, he found that it wasn't all that positive. Although patients were monitored, they weren't proactively managed. Decline was often treated as inevitable, and interventions occurred, but only after conditions had worsened. Sameer concluded that the system was not broken in the conventional sense – it worked as designed. But it was designed to respond, not to prevent.

In 2018, Sensasure was acquired, and Sameer moved into a leadership role at Essity. Despite having to pursue a new career path, his burning questions lingered. From his vantage point within a large organization, Sameer could see the scale of the problem more clearly, but also the difficulty of addressing it within existing structures.

Seeking a broader perspective, in 2021, Sameer applied to and was accepted into the Schwarzman Scholars program at Tsinghua University. For two years, he was surrounded by peers from across the world and exposed to discussions on global policy, economics, and long-term societal challenges. These experiences caused him to think differently about scale – not just in terms of capital, but in terms of human impact. It was during this period that Sameer found healthcare standing out as a system that was both critically important and deeply flawed.

During this time, he entered what he would later describe as a period of exploration. Sameer generated numerous ideas for new ventures, often working with a researcher, Emma, to gather early-stage data and validate assumptions. Some ideas seemed promising at first, only to lose momentum upon closer examination, while others were set aside and revisited months later with fresh perspective. He developed a habit of letting ideas sit, giving them time to reveal their strengths and weaknesses. Periodically, he would return to them, refining concepts, testing them against new information, and discussing them with a small circle of trusted advisors, including a friend in venture capital.

At the same time, his interest in healthcare became increasingly personal. Sameer began using advanced diagnostics to better understand and reduce his own health risks, including conditions like diabetes. Through mere behavioral changes and diagnostic testing, he was able to reduce his own diabetes risk by 300%. He thought, “if I was able to do this, imagine how many people could benefit from a system like this?” Soon after, his individual curiosity blossomed into a broader frustration about the healthcare system. He began exploring the emerging “proactive health” and longevity movement, studying how diagnostics combined with behavior change could extend not just lifespan, but healthspan. Through this, he arrived at a key realization: the tools to prevent disease already existed – biomarkers, testing, analytics – but they were fragmented, expensive, and difficult for individuals to use meaningfully. The opportunity, then, was not to invent something entirely new, but to bring these pieces together into a consumer-friendly, continuous health platform that combined diagnostics, insights, and action.

Sameer knew that Canada’s healthcare model, like many around the world, was designed to respond to problems rather than prevent them. He saw that proactive healthcare remained largely untapped, yet the tools to enable it already existed. The challenge was not invention, but integration: bringing together diagnostics, data, and behavior change into a single, scalable experience that individuals could engage with over time. It was this gap, between data and action, that became the foundation for NiaHealth. With a clearer sense of direction, Sameer turned his attention to building a team.

Building the team

Mike Goss's journey began in Alberta, Canada, where he enrolled at the University of Alberta to study Mechanical Engineering. Like many engineers, he was trained to think in terms of systems – how individual components interacted and how small inefficiencies could cascade into large-scale failures. During his early co-op experiences, including roles at companies like Syncrude and Endurance Energy, Mike was exposed to real-world systems that demanded both precision and adaptability.

After graduating in 2015, Mike moved into consulting, joining KPMG in Toronto. There, he worked across a range of industries, but increasingly found himself drawn to healthcare. As a consultant, he was tasked with analyzing organizations, developing strategies, and advising leadership teams on how to improve performance. He later joined Invictus Analytics and Strategy, advising executive-level clients across the public healthcare sector, and then Shoppers Drug Mart, where he became Director of Healthcare Strategy and Corporate Development – responsible not just for identifying opportunities, but for ensuring they were executed within a large, complex organization.

After four years, in August of 2023, Mike's path crossed with Sameer's in Toronto. Both had spent years thinking about healthcare and shared a core frustration: the system was full of intelligent strategies and abundant data, yet outcomes remained reactive, fragmented, and inefficient. More importantly, they saw an opportunity to build something that could actually work within its constraints while pushing it forward. All that was left was finding a technical expert that could build the product.

Halfway across the world in Asia, Saif Uddin Mahmud had been unknowingly preparing for exactly that challenge. Born in Bangladesh, he pursued Computer Engineering at the National University of Singapore, where he developed deep expertise in distributed systems and reliable software. During his time at university, he had the opportunity to go on exchange to the University of Toronto, where he was introduced to the North American tech and startup ecosystem.

After graduating during the peak of the COVID-19 pandemic in 2021, he worked at Thales Group as a software engineer developing big data analytics platforms for global train signaling systems, and later as a knowledge engineering consultant at Klinify where he helped significantly improve the efficiency of drug analysis across the ASEAN region. Alongside these roles, Saif began exploring entrepreneurship more directly – co-founding jezer0x, which focused on emerging technologies in health, AI, and Web3,

and later founding Sang Bio with the goal of democratizing access to personalized preventative health plans.

After moving to Toronto, when Saif met Sameer and Mike, it felt like everything he had been working toward was finally coming together in a single problem worth solving. By the time Sameer (CEO), Saif (CTO), and Mike (COO) began working together, the company had a clear direction. The platform could collect data, process it, and generate insights. But there was still a critical gap.

Tanya ter Keurs had spent her career navigating exactly that gap.

Her journey began in Canada, where she pursued a Bachelor of Science in Nursing at Thompson Rivers University and further deepened her foundation with a Master of Nursing as a Nurse Practitioner at University of Victoria. Tanya began her career in primary care, working closely with patients managing chronic conditions. Day after day, she found herself guiding individuals through uncertainty - helping them interpret symptoms, weigh trade-offs, and make decisions that would affect their long-term health. After a few years, she pivoted to virtual care platforms like Akira Health and Mednow, and seeking a challenge, eventually took on a leadership role revamping care delivery across four communities in one of British Columbia's first primary care networks. Initially, there was no budget, no rigid constraints, and full freedom to rethink how care could be delivered. "You rarely get to play around with government resources without a budget," she later reflected. "It was fun. It forced you to think outside the box." For a time, it felt like a rare chance to design systems from first principles. But as budgets, policies, and institutional constraints inevitably re-entered the picture, the limits of traditional healthcare became clear once again.

It was during this period that she was introduced to Sameer and the team through a mutual connection. Initially, she agreed to contribute ten hours a week, helping the team structure the early model while continuing other professional commitments. But it didn't take long for something to shift. The problem they were trying to solve was one she had been circling for years.

Within just two weeks, she made the decision to join full-time as Chief Clinical Officer.

Blue zones

With all four founders in place, the company began to take shape. Sameer, the chief executive officer, was in charge of fundraising and overseeing the company's strategic decisions. Saif, as chief technological officer, would lead the technological backbone. Mike, the chief operations officer, would ensure that operations could scale beyond a small pilot into a repeatable model. And Tanya, as chief clinical officer, committed to ground everything in clinical reality, ensuring that the insights would translate into meaningful, actionable care. Together, they started building the product as a platform that integrated advanced biomarker testing, aggregated health data, applied analytical models, and paired those insights with clinician guidance. The ambition was not to replace existing healthcare systems, but to augment them through a continuous layer of care focused on prevention and long-term health management rather than reactive treatment.

Even the name "Nia" reflected this vision. It was inspired by Sardinia, one of the world's original "blue zones" – regions identified for their unusually high concentrations of people living long, healthy lives. These communities shared common traits: consistent physical activity, whole-food diets, strong social connections, and an environment that naturally supported longevity. Sameer had spent time exploring these regions, and the experience left a lasting impression. What stood out was not just longevity, but quality of life: the idea that healthspan, not just lifespan, could be intentionally shaped.

By the end of 2023, what had begun as a set of questions, experiences, and converging perspectives had taken form. For the first few months, NiaHealth was bootstrapped, with the founders self-funding the initial build rather than bringing on outside capital. The founding team handled core development in-house, leaning on Saif's technical depth to construct the product's infrastructure and initial MVP without a dedicated engineering hire. The four of them worked day and night to build, market, fund, and test the product so they could gather customer feedback and iterate rapidly.

Early Vision

Early research that Sameer had done showed that roughly 100,000 Canadians, primarily health-conscious professionals, would be willing to spend about \$2,000 annually for proactive health services. The total addressable market, encompassing all adults globally interested in preventive healthcare, stood at roughly \$120 billion. Within Canada and comparable regions where NiaHealth could realistically operate, the serviceable available market narrowed to \$3.1 billion. For its immediate focus, the serviceable obtainable market of \$50 million represented urban, high-income early adopters most likely to

subscribe in the near term. These were professionals already primed for the category, familiar with executive health programs, and willing to pay a premium for proactive care. Executive health clinics like Medcan validated this assumption, maintaining long waitlists despite serving a limited geographic area. Initially, Nia's ideal customer profile skewed toward middle-aged men – the same demographic already paying for executive health programs. These individuals would experience Nia's preventive care firsthand, and over time, companies could extend the service across leadership teams, introducing accountability and structure to employee wellness at a fraction of the cost of traditional executive checkups.

That insight became central to NiaHealth's mission: to elevate health for humanity, starting with a next-generation, evidence-based health checkup. The platform combined blood testing, analyzing over 50 biomarkers, with additional VO₂ max fitness testing. But the value did not lie in the tests alone. Based on the results, Nia provided personalized plans designed to mitigate risk and optimize health outcomes, supported by one-on-one clinician consultations that helped members interpret insights and take meaningful action.

To support this growing service, NiaHealth developed a tiered subscription model billed annually. The entry-level Liteplan, priced at \$299 per person, was designed for those just beginning their health journey, offering annual blood testing across 40 biomarkers, longevity risk assessments, and a hyper-personalized health plan written by Canadian clinicians. For members seeking deeper insight, the Core plan at \$499 per person expanded coverage to 60 total biomarkers, 50% more than Lite, and included a live clinician consultation, making it the most popular tier. At the premium end, the Ultraplan at \$1,799 per person offered bi-annual testing across 120 biomarkers, combining an initial comprehensive panel with a mid-year follow-up of 60 biomarkers, catering to those committed to year-round health optimization. Together, the three tiers allowed NiaHealth to serve a broad spectrum of health-conscious Canadians, from the curious newcomer to the dedicated optimizer, while building a predictable, recurring revenue base.

By January 2024, the team launched an early, bare-bones version of the product. The initial cohort of around 20 members consisted largely of Sameer's close friends and family, who signed up for a simple annual subscription. Customers would complete routine blood draws once a year, with the option to add services such as VO₂ max fitness testing. To facilitate this, Nia partnered with Dynacare, a medical laboratory services company, leveraging its network across Canada. Members could either visit a clinic in person or opt for at-home blood draws, after which samples were processed by Dynacare and sent back to NiaHealth. The results were then compiled and delivered to members as static PDF reports on NiaHealth's website, clearly presenting their biomarker data but

offering limited interactivity. The real value of the experience came afterward. Once members received their reports, they would sit down with Tanya for a one-on-one consultation. In these sessions, she translated the numbers into meaningful summaries, contextualizing results, identifying potential risks, and providing personalized, actionable recommendations. In just about a month, NiaHealth made its services available across multiple Canadian provinces, including British Columbia, Alberta, Ontario, and Manitoba, marking an ambitious initial footprint for a young company operating in a highly regulated space.

While the data itself was powerful, the experience revealed immediate challenges. The entire system depended heavily on Tanya. She was not only conducting consults, but also ordering labs, following up with members, manually compiling reports, and even helping shape what the product should become. The “magic” of the experience, what made members feel understood and motivated to act, came almost entirely from the consult. Sameer and his team wondered if that level of personalization and impact could ever scale beyond a single clinician.

At the same time, the product itself was still evolving. PDFs were static and limiting, offering very little interactivity or ongoing engagement. The team began to recognize that if Nia was to become a continuous health platform rather than a one-time service, the experience needed to stand on its own, independent of any one individual. This led to an early push toward building a more dynamic dashboard, one that could translate complex medical data into intuitive, actionable insights. The goal was to make the product valuable even before a consult, and to prove that the consult model itself could be systematized and scaled across a growing clinical team.

New and improved

By the spring of 2024, the team began to see a path forward by rethinking how the product itself delivered value. The solution emerged through productization. Instead of relying on static PDFs and clinician-heavy interpretation, they built an interactive, data-driven dashboard that translated complex medical data into clear visual narratives with trends, benchmarks, and risk signals that members could engage with on their own. For the first time, the product itself began to carry part of the experience with consults evolving alongside it. What had once been spent explaining numbers shifted toward coaching and helping members understand how to apply insights in the context of their own lives. Over time, the dynamic changed. Users no longer arrived confused; they came prepared, curious, and ready to act. For Sameer, this was a turning point. It proved that clinical value could be scaled but by amplifying it through thoughtful product design.

As the product evolved, so did the team. Each founder's role became more defined in service of this new model. Saif led engineering, overseeing lab integrations, data pipelines, and security to ensure compliance with strict healthcare privacy standards. Sameer drove the company outward, leading fundraising, managing growth strategies, handling customer support, and even reviewing dashboards himself to ensure they remained intuitive and accessible. Mike translated ambition into execution, building operational systems and analytics frameworks that could support scale, while working closely with Tanya to ensure the product balanced medical accuracy with usability. Tanya designed clinical protocols, trained new hires, and maintained quality across the experience, transforming what had once been a static report into an interactive, coaching-driven system that empowered members to take action independently.

As a result, growth began to follow. The first 50 customers were acquired through direct-to-consumer channels, though always with a longer-term B2B vision in mind. By the time the company reached its first 200 members, that mix had begun to shift, with early B2B cohorts joining alongside individual users. At the same time, the product itself deepened. Nia expanded its diagnostic capabilities, partnering with Dynacare and LifeLabs to support lab testing at scale. The platform incorporated more biomarkers, introduced add-on services like continuous glucose monitoring (CGM) and gut microbiome testing, and began supporting a growing base of engaged users.

As the product began to take shape and early traction validated the model, the team turned their attention to building the resources needed to scale. By the end of 2024, NiaHealth officially stepped out of stealth and raised a \$2.5 million CAD pre-seed round, backed by investors including Version One Ventures, ScaleGood Fund, Garage Ventures, and Union Capital, alongside a group of angel investors. The funding allowed the company to bring its vision to market: launching a proactive health platform centered around advanced diagnostics such as biomarker testing and wearable-integrated insights, aimed at identifying early risks like pre-diabetes, nutrient deficiencies, and other preventable conditions.

But with growth came a familiar challenge: the human element. For the first year, Tanya had been the sole clinician delivering consults. By early 2025, it became clear that this model would not hold. The team began expanding the clinical function, hiring and training nurse practitioners to replicate the depth and quality of Tanya's approach. Within a year, the team had grown to nine to ten NPs, supported by two registered nurses specializing in specific areas. Scaling the consult experience, without losing its "magic", became one of the company's most important operational priorities.

NiaHealth was no longer just a promising idea; it was becoming a system that could scale.

Competition?

As Nia gained traction, the competitive landscape began to take shape. International players like Function Health prepared to enter the Canadian market, and domestic startups such as MyHealthspan and Jack Health explored adjacent offerings. On the other hand, wearable companies like Apple Health, Garmin, and Oura hinted at expanding into preventive health services. Even online health providers such as Amazon Health and GPT Health were beginning to blur the lines between consumer wellness and medical guidance. Despite the market crowding quickly, Sameer still believed Nia was different.

Nia's advantage rested on three clear pillars: first, being a first mover in Canada gave the team credibility, time to refine workflows, and the opportunity to build trust with members before international players could localize their offerings. The second was the human-in-the-loop element with clinician consults. User feedback consistently reinforced this point: while members found the dashboard informative, the one-on-one consults elevated the experience from good to exceptional. Members felt seen and understood in a way that generic or fully automated platforms simply could not replicate. Lastly, Nia's personalized dashboard translated complex biomarker data into actionable, contextualized insights, bridging the gap between knowing what to do and actually doing it. Primary care often leaves patients knowing what they "should" do, like eating better and exercising more, but without tools to integrate these recommendations into daily life, it becomes very difficult to implement. Nia's dashboard visualizes health metrics, contextualizes them, and equips members to act by understanding each person's story, goals, and constraints, turning insights into practical, sustainable steps. Members reported that seeing their data in an understandable format often drove change more effectively than traditional clinical visits.

By positioning itself at the intersection of preventive healthcare, human coaching, and technology-enabled insights, Nia created a defensible niche. Competitors could replicate a dashboard, but replicating the combination of clinically accurate data, personalized coaching, and a trusted, human-centered experience proved much harder. This differentiation allowed Nia to maintain high satisfaction and retention while preparing for future scale, even as new entrants eyed the growing preventive health market in Canada.

Pivots, partnerships, and growing pains

As the product matured, Sameer decided to test both B2B and D2C strategies to see which was performing better. He led partnerships with local executive health clinics, which validated scalability and retention beyond founder-led delivery. On the other

hand, the D2C channels offered rapid feedback and opportunities for Nia to iterate on engagement, marketing, and dashboard functionality. Sameer found that users often came through referrals, creating a tight feedback loop that informed product improvements. This dual approach ensured Nia could reach both companies seeking to invest in employee wellness and individual consumers pursuing personalized preventive care.

Because both strategies offered advantages, Sameer decided to double down on two pivotal strategic decisions targeting B2B and D2C motions simultaneously. For the first few months of 2025, Nia pursued a partnership with first responder organizations in Canada. Organizations like the Toronto Fire Service and Toronto Police Service were in need of affordable, structured health solutions, and Nia needed B2B partnerships that could compound over time. It was an unconventional choice, as first responders were not the typical profile for biomarker-based health programs. As a result, Sameer put in significant effort to make it work. He personally prepared every dashboard and had Tanya lead each consult, investing hours in labor-intensive, manual processes. The first cohort generated no immediate revenue, prompting internal doubts about whether the model could ever justify the effort. Yet as workflows improved and early feedback confirmed strong member satisfaction, the motion began to take off. First responders engaged deeply with the product and demonstrated measurable health behavior improvements, transforming what had been an uncertain pilot into a scalable proof of concept.

While the first strategy focused on B2B, the second major decision pivoted toward accelerating the direct-to-consumer motion for the second half of 2025. To drive this, Nia partnered with TELUS, one of Canada's largest telecommunications and health services companies, leveraging its expansive reach to gain both distribution scale and market credibility. In parallel, Nia brought on a Chief Marketing Officer to lead and formalize the D2C function, recognizing that consumer acquisition in healthcare required dedicated expertise and a more structured approach than the team had previously invested in. The company also undertook a brand refresh, sharpening how NiaHealth communicated its value to individual consumers and ensuring the product's positioning reflected the depth and quality of the experience it delivered. Together, these three moves formed the backbone of Nia's D2C push. The partnership with TELUS promised meaningful visibility and potential volume, but it also required navigating corporate alignment, regulatory nuances, and member engagement simultaneously. These were not mere operational hurdles; they carried real stakes for the company's reputation and its ability to sustain the growth it was working so hard to unlock.

In the midst of these pivots, Sameer confronted two defining operational challenges that would test his judgment as a founder and shape how NiaHealth functioned as a business.

The first was securing a reliable lab partner. With only two major laboratories operating at scale across Canada, LifeLabs and Dynacare, Sameer had limited room to negotiate and even less room for error. Establishing the lab draw processes, reporting workflows, and compliance standards required to support a growing member base was a painstaking effort, and the concentrated nature of the Canadian lab ecosystem meant that a failed partnership could have stalled the entire operation before it had a chance to prove itself.

The second challenge was subtler but in many ways more consequential. Sameer had to reconcile three fundamentally different cultures operating under the same roof. The tech world's instinct to move fast and break things collided directly with the realities of healthcare, where the cost of moving carelessly was not a missed target or wasted ad spend, but real harm to real people and the professional livelihoods of the clinicians he had brought onto his team. Learning to hold these tensions together, pushing for speed and scale while never losing sight of the responsibility that came with operating in healthcare, became one of the most important leadership lessons of Sameer's journey. It forced him to develop a more nuanced decision-making framework, one where ambition and caution were not opposites to be traded off, but disciplines to be held simultaneously.

Despite the massive changes, the company continued to raise capital, securing a \$5.75 million CAD seed round in June 2025. With total funding surpassing \$8.25 million, NiaHealth now had the financial backing to invest more aggressively in product development, clinical expansion, and go-to-market strategies, setting the stage for its next phase of growth.

By the end of the year, Sameer's leadership had produced measurable wins across the organization. Engagement, retention, and satisfaction increased steadily as the product matured and the clinical team grew into a capable, scalable unit that could deliver consistent quality without depending on any single individual. Marketing processes became both legally compliant and meaningfully effective at reaching and converting the right audiences. Members reported tangible improvements in health literacy and behavior, offering validation not just of the product itself but of the broader growth strategy the team had committed to. Sameer and his co-founders had achieved the system and processes needed to support the next phase of growth.

What is NiaHealth today

The combination of technology and human guidance began to translate into tangible, measurable results. What had once been a manual, clinician-heavy process evolved into

a scalable system without losing its impact. Externally, member satisfaction remained exceptionally high, with Net Promoter Scores consistently exceeding 90% and Sean Ellis scores above 40% across channels. Growth accelerated as well – monthly member additions increased from just 25 in the early days to over 300, while average order value rose from \$700 to more than \$900. On the direct-to-consumer side, revenue expanded rapidly, growing from approximately \$10K per month to \$250K per month. Early B2B partnerships, combined with strengthening D2C traction, reinforced a critical signal: Nia could deliver both outcomes and retention at scale. By the end of 2025, the company had reached approximately \$5 million in annual recurring revenue, marking a significant milestone in its journey.

Internally, the transformation was just as striking. Processes that once took over eight hours per report were reduced to under twenty minutes, and delivery timelines improved from up to seven business days to fewer than five. The clinical team, which initially consisted of a single full-time practitioner, expanded to four or more full-time equivalents, enabling the company to support a growing member base while maintaining quality. Beyond efficiency, these operational gains were what made scale possible.

As a company, NiaHealth had grown from four co-founders to approximately 30 employees in just one and a half years. The clinical team, overseen by Tanya, consisted of a research function and a growing group of nine to ten nurse practitioners supported by two registered nurses. Marketing, working closely with Sameer, drove both B2B and D2C acquisition efforts. Saif led an engineering team of five, collaborating closely with a product manager who coordinated with two designers. Mike anchored a lean customer experience and operations team responsible for retention, engagement, and expansion. Across all functions, Saif actively encouraged the adoption of AI tools, fostering a culture where nurse practitioners, designers, and operations staff alike contributed to core product thinking and ideation, blurring traditional role boundaries and accelerating the pace of iteration.

Yet, even as the numbers improved, the team recognized that the next phase of growth would require more than optimization. Looking ahead to FY26, the focus shifted toward closing what they called the “behavioral loop” – ensuring that members didn’t just receive insights, but acted on them, tracked progress, and built sustainable habits over time. The ambition was bold: grow to \$20 million in ARR while maintaining retention above 50%. Achieving this would require continued investment in clinical capacity, deeper refinement of the dashboard experience, and a relentless focus on translating insights into personalized, actionable guidance.

From a small pilot with a handful of users to a structured, repeatable model, NiaHealth had demonstrated that technology could amplify clinicians, consults could drive real behavior change, and members could actively engage with their health in a meaningful way. The mission ahead was clear: to transform proactive healthcare from a one-time interaction into a continuous habit, and in doing so, unlock measurable impact at scale.

The rise of the growth dilemma

The growth dilemma at Nia emerged gradually, shaped by months of operational experience and strategic reflection. In the company's early days, Sameer's personal network drove rapid momentum. Personal connections, corporate partnerships, and early evangelists brought in the first wave of members, providing initial validation and confidence. As this pipeline matured, however, growth began to plateau. Despite hiring a CMO to accelerate acquisition, D2C sales cycles proved far longer and more complex than expected, requiring six to twelve months of sustained engagement to build trust, educate prospects, and guide them through a healthcare experience unlike anything they had previously encountered. B2B partnerships, meanwhile, delivered growth in bursts. Signing a company or a cohort of first responders could bring in hundreds of members at once, but these wins were episodic and difficult to predict. Together, these dynamics created a core challenge: neither channel provided a reliable, repeatable growth engine.

This reality pushed Sameer into a difficult internal reckoning. One school of thought within the team held that aggressive investment in marketing would unlock growth by expanding awareness and driving more leads into the funnel. The opposing view argued that awareness was not the constraint, and that the product experience itself needed to deliver more actionable value before conversion could meaningfully improve. The regulatory environment complicated this debate further. Unlike traditional tech companies, Nia could not afford to “move fast and break things.” Missteps carried real risks, including regulatory penalties and compromised patient safety. This created a fundamental tension: growth demanded speed, while healthcare demanded precision and caution.

If Nia chose to double down on D2C acquisition, the company could potentially increase top-of-funnel activity and generate more leads. Over time, this might lead to higher absolute conversions, particularly as brand awareness improved. However, the underlying friction in the user journey would remain. Long sales cycles, low initial engagement, and limited perceived value before clinician interaction would continue to constrain conversion rates. This would likely drive up CAC, pressure ROAS, and result in inefficient growth. Without addressing the core product experience, scaling D2C risked

amplifying inefficiencies rather than resolving them by creating growth in volume, but not in quality, retention, or long-term value.

In contrast, the team began to explore an alternative path: building a product so compelling that growth would follow organically. Both qualitative and quantitative data showed that members deeply valued the human element and personalized guidance. While users often described the dashboard as “fine,” their experience transformed after clinician consults, where they felt understood, supported, and given clear direction. Engagement data reinforced this pattern: members who received actionable insights or participated in consults were significantly more likely to renew, refer others, and stay engaged.

This led to a new hypothesis: if Nia could deliver a consistently “magical” experience, one where members understood their health, took action, and saw measurable improvement, growth would follow across both D2C and B2B channels. Importantly, “magic” extended beyond interface design. It meant delivering real outcomes, continuous support, and adapting to members’ evolving needs. The ambition was to transform healthcare from a one-time interaction into an ongoing practice, becoming a platform users could not imagine living without.

To better inform this decision, Sameer and the team turned to both qualitative and quantitative insights. He observed consults, onboarding flows, and re-engagement journeys, identifying moments where users felt motivated versus disengaged. In parallel, the team analyzed behavioral data (logins, follow-ups, redraws, conversions, and campaign engagement) linking these actions to retention and outcomes. During user interviews, Sameer also uncovered a critical pain point: many users felt that the product created value only once a year, around the time of the annual blood draw, after which engagement dropped off. Users expressed a desire for more continuous value throughout the year, rather than a one-time insight from the annual blood draw. This reinforced a broader pattern: members stayed when they were supported year-round in taking action, tracking progress, and seeing tangible improvements in their health. Conversely, engagement dropped when users felt like the product delivered value only once, without enabling continued progress. This pointed to a deeper issue: the experience lacked continuity.

From this, Sameer worked with Saif and the product manager to develop the behavioral loop concept, where members join, test, gain insights, take action, track progress, and repeat. The team hypothesized that strengthening this loop could transform Nia from a transactional service into an ongoing, habit-forming platform. Potential initiatives included expanding follow-up consults, syncing Apple health wearable data, introducing communities for Nia users, and improving progress visibility. Member research

supported these directions: 57% of users expressed interest in better progress tracking and habit support, 38% prioritized ongoing care, and 20% had already connected wearable devices without prompting.

Despite these insights, the path forward remained uncertain. Nia had set ambitious targets, such as 15% month-over-month growth, strong ROAS and CAC benchmarks, and a trajectory toward \$20M ARR, but current performance lagged behind expectations. Each strategic option presented trade-offs between speed, efficiency, and long-term value creation.

The dilemma was not merely operational but existential. Pursuing D2C growth risked inefficiency without deeper product value. Focusing solely on product risked slowing momentum in a competitive market.

Sameer's role became one of navigating this ambiguity and involved balancing data with intuition, short-term metrics with long-term vision, and growth ambitions with the realities of a regulated healthcare environment. The decision remained unresolved, but the stakes were clear: the path Nia chose would define not only its growth trajectory, but its ability to deliver on its mission of transforming proactive healthcare into an enduring, everyday practice.

What's next?

Sameer stood in the conference room, the whiteboard full of diagrams mapping the behavioral loop: Join. Test. Insights. Act. Track. Progress. The numbers glared back at him - Revenue on Ads Spent (ROAS) hovering just above 2.5, Customer Acquisition Cost (CAC) inching under \$300, Annual Recurring Revenue (ARR) climbing toward \$6.14M - but the metrics only told part of the story. He glanced around the room at his co-founders. The decision did not need more data; it needed alignment. They had two solid options. Now they just had to decide which one to bet on.